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Title: Henoch-Schonlein Purpura (HSP)

Statement: This guideline is made to guide pediatrician/OMSB residents, the approach of a child with IgA vasculitis formerly known as Henoch-Schonlein purpura.

Introduction: IgA vasculitis, formerly known as Henoch Schonlein Purpura is the commonest systemic vasculitis in childhood. It is a small vessel vasculitis with IgA-dominant immune deposits that typically involves the skin, gut and kidney, and is associated with arthralgia and/or arthritis.

Diagnostic criteria of IgA vasculitis, formerly known as Henoch Schonlein Purpura:

Purpura or petechiae is (**mandatory**) in a predominantly lower limb distribution with and at **least 1 of the 4 following criteria:**

1. Diffuse abdominal pain.
 2. Histopathology showing IgA deposition.
 3. Arthritis or arthralgia.
 4. Renal involvement (hematuria or proteinuria).
- In severe abdominal pain, **an ultrasound** should be performed to exclude intestinal intussusception.
 - A **skin biopsy** including specific immunofluorescence staining for IgA should be performed in case of **atypical rash** to exclude alternative diagnoses. It is not indicated in a patient with the typical purpuric skin rash on lower limbs and buttocks.
 - Renal involvement should be investigated using urine analysis (hematuria and urine protein creatinine ratio).
 - Pediatric nephrologist should be consulted if there is moderate proteinuria (urine protein creatinine ratio is > 100 mg/mmol and/or impaired renal function test. **A renal biopsy should be performed in severe proteinuria (>250 mg/mmol.**

- Patients with persistent nephrotic range proteinuria or renal insufficiency should be referred to a pediatric nephrologist for renal biopsy.
- Henoch Schonlein Purpura HSP vasculitis rash disappear late it may take up to 6 -8 weeks.

European consensus-based recommendation for treatment of HSP.

1. Analgesia.

- NSAID (Ibuprofen /Naproxen) be prescribed for arthralgia/arthritis for 2 weeks.
- NSAIDs are not contraindicated if renal function is normal.
- Adequate analgesia can be prescribed for mild abdominal pain.

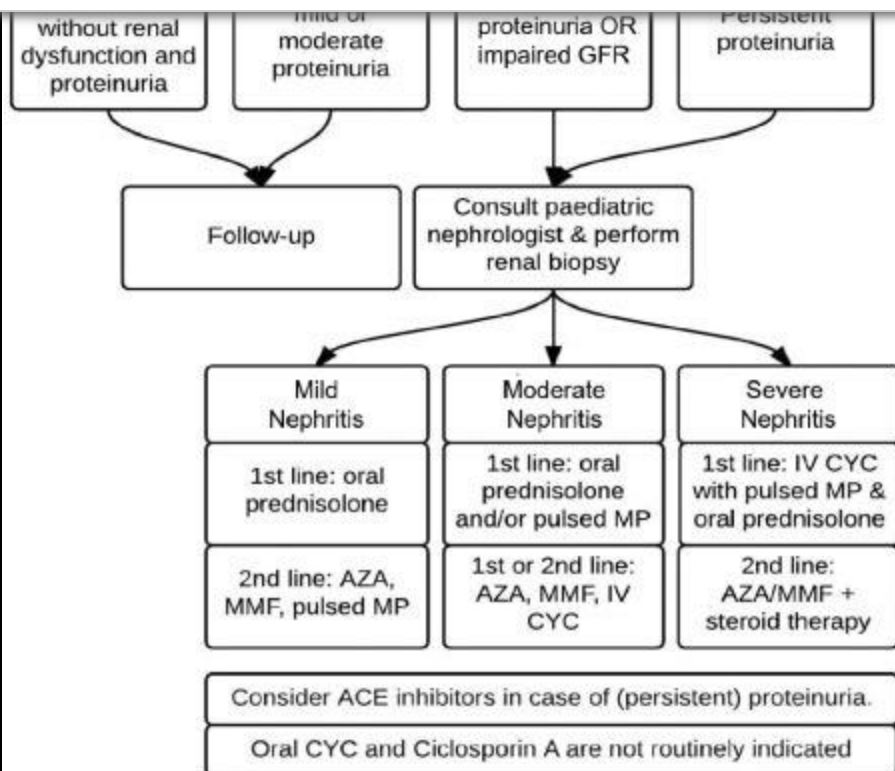
2. Corticosteroid treatment is indicated in the following cases:

1. Severe abdominal pain (after intestinal intussusception has been excluded).
 2. Gastrointestinal bleeding (hematemesis /melena).
 3. Orchitis.
 4. Severe arthritis not responding to NSAID.
- Prednisolone dose (1- 2 mg/kg/day) for 1weeks with weaning plan over 2-3 weeks. If child is not tolerating oral intake or vomiting intravenous Methylprednisolone (1-2 mg/kg/day)can be given for few days till patient is tolerating oral medication then it can be converted to oral prednisolone.
 - Methylprednisolone pulse (10-30 mg/kg) with a maximum of 1000 mg/day for three consecutive days may be considered in severe cases of abdominal pain.
 - ER consultant / general pediatric consultant/ pediatric specialist doctor in region hospital can prescribe steroid if indicated without consult pediatric rheumatology team.

Pediatric rheumatology team to be consulted in the following situation.

1. Severe case of HSP not responding to steroid.
2. In case a multi-disciplinary team MDT approach is needed. Example in case of abdominal pain with GI bleeding (hematemesis, bloody stool), hematuria, nephrotic range proteinuria, hypertension, derange renal function, arthritis and subcutaneous angioedema.
3. HSP patient with positive anti-nuclear antibody ANA and low complements C3, C4.

Guideline for the management of IgA vasculitis associated nephritis



References:

Title of book/ journal/ articles/ Website	Author	Year of publication	Page
Pediatric rheumatology handbook	Helen Foster	2012	179
Pediatric rheumatology text book	Cassidy	5 th edition 2005	496
European consensus -based recommendation for diagnosis and treatment of Ig A vasculitis.	Seza Ozen et al.	2019	1607

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